Family Caregiver Alliance®

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RE: CMS-0042-NC – Family Caregiver Alliance (FCA) Comment on Health Technology Ecosystem RFI

INTRODUCTION

Family Caregiver Alliance (FCA) is a national leader in caregiver support, serving over 1 million caregivers annually. As the longest-standing nonprofit dedicated to family caregivers, our work offers a replicable blueprint for national digital health transformation and value-based care delivery models that elevate caregivers as essential partners.

FCA appreciates the opportunity to comment on the Health Technology Ecosystem Request for Information (RFI). FCA's 45-year legacy includes direct services, policy leadership, and careful adoption of technology innovations that center family caregivers. As a trusted intermediary between families and systems, we bring a pragmatic, family-informed lens to digital health, with an emphasis on safeguards, responsible innovation, and collaboration across sectors.

Policy Context: Urgency and Opportunity

Federal and state policies are rapidly advancing toward integrated, value-based models that center on the caregiver-recipient dyad. More than 5.2 million individuals are enrolled in Dual Special Needs Plans (D-SNPs) nationally, many of whom are older adults with multiple chronic conditions and rely on unpaid family caregivers. A recent Medicaid and CHIP Payment and Access Commission (MACPAC) analysis (June 2025) recommends stronger standards and federal leadership to ensure the effective integration and support of dually eligible beneficiaries and their caregivers. The Center for Medicare and Medicaid Innovation (CMMI) has refreshed its strategy to emphasize prevention, individual empowerment, choice, competition, and cost savings for the federal taxpayer. These shifts are not theoretical. They reflect a bipartisan evolution in healthcare delivery, reinforced by federal legislation, Centers for Medicare & Medicaid Services (CMS) demonstration models, and recent guidance from MACPAC and CMMI.

FCA'S MODEL: MODERNIZING CARE, LOWERING COSTS, AND IMPROVING EXPERIENCE

FCA has developed an integrated, caregiver-centered model that enhances care coordination and support systems, improving outcomes for both caregivers and care recipients. Our approach leverages a configurable, standards-aligned platform that enables real-world care coordination between family caregivers, clinical providers, and community-based organizations.

The model facilitates dyadic engagement through adaptable workflows aligned with national standards, supporting shared decision-making and coordinated care management without requiring proprietary dependencies or direct electronic health record (EHR) integration. It also enables structured consent processes and the integration of social and functional data, aligning with CMS's vision for a more connected health system.

FCA's infrastructure is built for interoperability from the ground up, leveraging national interoperability protocols and Open APIs (Application Programming Interfaces) to enable integration across clinical and community care systems. Unlike legacy systems that rely on proprietary portals or closed data environments, FCA's model is designed to extend existing care infrastructure, not replace it, making it especially well-suited for partnership with Medicaid and Medicare plans serving complex, high-need populations.

Our work is particularly responsive to the needs of rural, multilingual, and underserved communities. By combining digital tools with culturally responsive, relationship-based support from community-based teams, FCA helps reduce avoidable utilization, delay institutionalization, and improve adherence for older adults and their caregivers. These outcomes mirror those demonstrated by nationally recognized care navigation models, while also addressing gaps in workforce reach and caregiver inclusion.

FCA's interdisciplinary team-based model also contributes to system capacity by expanding the role of community health workers, social service professionals, and peer support personnel in digitally supported care environments. This approach provides a practical and scalable solution for advancing CMS's goals related to value-based care, person-centered innovation, and delivery system transformation.

MODERNIZING INTEROPERABILITY: CONSUMER ACCESS AND INNOVATION

CMS and the Office of the National Coordinator for Health Information Technology (ONC) have made significant progress through the 21st Century Cures Act, the Interoperability and Patient Access Final Rule (85 FR 25510), and the Trusted Exchange Framework and Common Agreement (TEFCA). However, the digital health ecosystem remains constrained by legacy Electronic Health Record (EHR) vendors and institutional gatekeepers, which limit consumer and caregiver access to health data and slow innovation.

RECOMMENDATIONS[1]

1. **(VB-1, PC-2) Recognize the caregiver-recipient dyad as a foundational unit of care**
   Establish the caregiver-recipient dyad as a core construct in CMS digital health policy, certified health IT standards, and value-based care models, reflecting the design of emerging demonstrations such as the GUIDE model and D-SNP requirements.

2. **(PC-4, PC-12) Expand CMS APIs to support caregiver access and real-world workflows**
   Enhance CMS APIs (e.g., Blue Button 2.0 and FHIR-based endpoints) to include caregiver-reported data, functional and social needs assessments, appointment data, and care transition records. Ensure this data is accessible to authorized caregivers and community-based partners.

3. **(VB-2, PC-6) Fund and scale caregiver-inclusive digital models**
   Prioritize CMS Innovation Center demonstration models and payment reforms that enable the deployment and evaluation of digital dyadic care coordination platforms, including integrated care coordination solutions that center caregivers as part of accountable, community-based care teams.

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[1] *Note: Citations such as VB-1, PC-2, etc., refer to section identifiers within the CMS Request for Information on the National Health Technology and Data Ecosystem for Health Equity and Outcomes Improvement (CMS-0042-NC), published in the* Federal Register *on May 16, 2025. The full RFI is accessible at:* https://www.federalregister.gov/documents/2025/05/16/2025-08701/request-for-information-health-technology-ecosystem

4. **(PC-5, TD-9) Strengthen the digitally enabled aging and caregiver workforce**
   Invest in the training and integration of community-based organizations, community health workers (CHWs), and digitally fluent caregiving teams to support longitudinal engagement and care coordination, especially in underserved and aging populations.

5. **(TD-4, TD-8) Require an open, modular architecture for all certified health IT**
   Mandate that certified health IT and payer platforms offer open, HL7 FHIR-compatible APIs that enable integration with nontraditional actors—such as caregiver-facing tools, mobile apps, and CBO-run platforms—without dependency on proprietary vendor systems.

6. **(TD-9, PC-7) Accelerate certification and funding pathways for nonprofit–health tech collaborations**
   Develop fast-track technical assistance, certification guidance, and grant programs that support nonprofit–digital health partnerships, particularly those focused on complex populations, such as dual eligibles, older adults, and rural caregivers.

7. **(VB-1, VB-2, PC-12) Link digital health standards to value-based metrics**
   Tie certification, interoperability, and API investment requirements to measurable outcomes, such as caregiver engagement, reduction in avoidable hospitalizations, medication adherence, and caregiver-reported experience, ensuring that digital adoption translates to better care.

A NOTE ON IDENTITY CREDENTIALS AND DATA PRIVACY

FCA strongly supports the principles of data transparency, access, and interoperability. However, we urge CMS and ONC to proceed cautiously in pursuing universal identity credentials or digital IDs as the backbone of health data exchange. The risks of unintended linkage of sensitive information, mainly if accessed or downloaded outside the protections of government systems, pose serious concerns. Widespread data exposure and unauthorized use, as seen in recent cases of private entities bypassing safeguards, must be addressed before further expanding the use of universal digital credentials across systems.

CONCLUSION

We encourage CMS and ONC to leverage organizations like FCA, whose decades of community-based innovation, data interoperability, and real-world care coordination provide proven models for national scaling.

FCA stands ready to serve as a national implementation and advisory partner to CMS and ONC in advancing caregiver-centered digital health models. We welcome participation in pilot programs, certification advisory panels, and federally supported technical assistance efforts to scale digital caregiver navigation across Medicare and Medicaid populations.

CONTACT INFORMATION

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REFERENCES

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2. Medicaid and CHIP Payment and Access Commission (MACPAC). *June 2025 Report to Congress on Medicaid and CHIP*. Washington, DC.

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DISCLAIMER

This comment submission reflects FCA's policy recommendations and high-level strategic priorities. Proprietary technical, operational, and contractual components of CareNav Health are not included in this public filing.